MateriaLens Safety & Limitations

Product boundaries, refusal behaviour, privacy controls and known limitations

1. Intended use

MateriaLens is proposed as a research and reference-retrieval assistant for qualified homeopathic practitioners working with a defined, permission-checked corpus of Materia Medica and repertory/reference texts. It helps locate, compare and inspect what the indexed sources say. It does not establish whether a source claim is clinically effective or medically appropriate.

2. Explicit non-goals

- It does not diagnose a patient, recommend a remedy, prescribe treatment, determine dosage or replace clinical judgement.
- It does not establish clinical efficacy, safety or suitability from the fact that a historical source contains a statement.
- It does not answer from general model knowledge when the indexed corpus does not support an answer.
- It does not silently resolve contradictions between sources or present a partial search result as consensus.
- It does not guarantee that every relevant passage in the corpus has been retrieved.

3. Risk register

Risk	Potential impact	Control / mitigation	Residual limitation
Unsupported generation	A plausible answer may include claims absent from the source.	Constrain generation to retrieved context; require claim-level citations; audit groundedness; abstain when evidence is inadequate.	Automated checks may miss subtle unsupported claims; human review remains necessary.
Incorrect citation	Reader may be unable to verify a claim or may trust the wrong passage.	Use stable passage IDs and source metadata; validate citation IDs; sample-check source location and support.	A valid link does not itself prove that the passage supports the claim.
OCR corruption	Older scans, typography or abbreviations may be misread and retrieved incorrectly.	Record source quality; sample-check OCR; preserve page references; flag low-confidence text and repair before indexing.	Some OCR errors may remain, especially in degraded scans.
Chunking loss	A symptom, modality, rubric or remedy list may be separated from its context.	Chunk on structural boundaries where possible; retain section metadata; test remedy abbreviations and rubric context.	Best chunking settings depend on actual corpus structure and need empirical testing.
Conflicting sources	The output may imply agreement where sources differ.	Show source-by-source evidence and explicit conflict/missing-evidence notes.	The product cannot determine which source is clinically correct.
Sensitive case details	User-entered health-related information may be exposed or retained unexpectedly.	Minimize data collection; do not retain case details beyond the active session unless explicitly saved; define access and deletion controls.	Privacy guarantees depend on the eventual hosting, logging and vendor configuration.
Out-of-scope clinical request	A research tool could be mistaken for a clinical decision-maker.	Refuse diagnosis/prescribing requests and clearly state the tool's research-only purpose.	A disclaimer alone is not sufficient; behaviour must be tested.
Corpus rights / stale content	Unauthorized content use or outdated editions may undermine trust.	Check access and reuse rights; track edition, version/hash and ingestion date; re-index approved changes.	The source inventory and refresh owner must be confirmed before launch.

4. Refusal and low-confidence behaviour

A. Insufficient evidence

“I couldn’t find sufficient supporting evidence in the indexed sources to answer this reliably. I won’t infer an answer from outside the corpus. You can narrow the question or choose another authorized source.”

B. Diagnosis or prescribing request

“MateriaLens supports source-based research only. It can show what the indexed references say, but it cannot diagnose a patient or recommend or prescribe treatment. Please use your professional clinical judgement and appropriate clinical processes for decisions about care.”

C. Ambiguous query

“Could you specify the symptom, modality, concept or source you want to research? I’ll search the indexed references once the question is clear enough.”

D. Conflicting evidence

“The retrieved sources differ on this point. I’ve shown the relevant passages separately; MateriaLens cannot determine which source is clinically correct.”

5. Human review and escalation

The primary human reviewer is the qualified practitioner using the tool. MateriaLens should not transfer or decide a case automatically. When the request requires clinical judgement, when sources conflict materially, or when repeated searches return weak evidence, the product should make the limitation visible and ask the practitioner to review the original sources or follow their established clinical process. If a product support workflow is later introduced, only the minimum necessary query and diagnostic metadata should be shared, and the user should be told what is being shared.

6. Privacy and data handling

- Collect only information required to execute the search. Avoid asking users to enter identifiable patient details.
- By default, user-entered case information should remain in the active session and not be retained unless the user explicitly saves it.
- Before launch, document where queries, feedback and logs are stored; who can access them; retention periods; deletion processes; and any third-party model processing.
- If account deletion is supported, remove or irreversibly de-identify account-linked saved queries and user data according to the documented retention policy; disclose any backups or legally required retention.
- Never commit API keys, user data, private case notes or licensed source files to the public repository.

7. Known limitations

- The corpus may be incomplete, and retrieval can miss relevant passages.
- Search quality depends on source quality, OCR, metadata, chunking and retrieval settings.
- Initial chunk size, overlap, Top-5 retrieval and model choices are design assumptions to be evaluated, not proven optimal settings.

- A citation verifies provenance, not clinical truth or efficacy.
- Performance claims and release targets must remain labeled as proposed until supported by recorded tests.

8. Pre-release safety checklist

☐ All source files have documented access/reuse status.

☐ Citations resolve to the correct source and location.

☐ Insufficient-evidence cases abstain instead of guessing.

☐ Diagnosis/prescribing and prompt-bypass requests are tested.

☐ Conflicting-source cases are shown without inventing consensus.

☐ OCR and chunking issues have representative test coverage.

☐ Data retention, account deletion, logging and vendor processing are documented.

☐ Repository contains no credentials, private data or unauthorized source texts.